

Clinical Management Handbook

Enhanced from Clinical_Guidelines_Compact.docx.pdf using workbook condition sheets

Audience: Clinicians in training and early practice

This handbook presents each condition as a practical bedside pathway in natural language.

Content has been expanded with sheet-level medications, treatment options, investigations, and decision points.

Always apply local policy, supervision, and patient-specific clinical judgment.

Condition 1: PV BLEEDING

Source rows integrated: 25

Presentation: Per Vaginal Bleeding. Clinical features: with sign of shock(Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg. Examination findings: NO Local bleeding lesions. Initial investigations: FHG + Pelvic U/S as you continue fluid resuscitation. Key result pattern: Normal FHG. Additional tests: Hormonal profile, U/E/C,. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause; Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause , ITP, VWD,; Probable dx: SHOCK with Abnormal Uterine Bleeding ? Cause, Anemia; Probable dx: Abortion with SHOCK; Probable dx: Abortion with SHOCK, DIC; Probable dx: APH with SHOCK; Probable dx: APH with SHOCK, DIC; Probable dx: with Abnormal Uterine Bleeding ? Cause.

Initial management pathway: See appendix (x) for hypovolemic shock resuscitation; Reassure patient, Start normal saline if actively bleeding as you consult. Operational plan points include: Admit + Gynecological Review; Admit for Gynecological Review IF ACTIVELY BLEEDING If not bleeding see as OP & TCA in GOPC.

Medication and treatment options documented in the workbook include the following regimen examples: Treatment: Tranexamic acid 1g IV tds, IV Ceftriaxone 50mg/kg, IV Metronidazole 7.5mg/kg. Formulary examples/brands in source: Hemsamic 50mg inj, Rocephin 1gm,Flagyl infusion. Treatment: OUT PATIENT:Tranexamic acid 1g PO tds, OP Cefuroxime 50mg/kg,PO Metronidazole 7.5mg/kg, IN PATIENT Tranexamic acid 1g IV tds IV Ceftriaxone 50mg/kg, IV Metronidazole 7.5mg/kg. Formulary examples/brands in source: Hemsamic 500mg caps, Axacef 500mg tab, Flagyl 400mg tablets.

Condition 2: hotness of body

Source rows integrated: 196

Presentation: Hotness of Body. Clinical features: No other sign. Initial investigations: FHG + malaria AG. Key result pattern: Normal FHG+ Malaria negative. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: Acute febrile illness R/O viral, atypical infections; acute bacterial infection; acute viral infection; anaemia + acute febrile illness; Malaria; malaria+ bacterial infection; malaria + viral infection; severe malaria.

Initial management pathway: Expose patient , tepid sponge; Expose patient , tepid sponge , secure an

IV line neck support, reassure the relatives; Expose patient , tepid sponge , secure an IV line neck support, reassure the relatives start ringers lactate as you consult; Expose patient , tepid sponge , secure an IV line neck support, reassure the relatives, start with normal saline as you consult; Expose patient , tepid sponge , secure an IV line neck support, reassure the relatives start with normal saline as you consult. Operational plan points include: see as OUT PT ,TCA 7 days; admit patient for further work up and possible transfusion; see as an outpatient and TCA 3 days; admit patient for parenteral treatment; admit patient for parenteral treatment and further work up; Admit sedate(with midazolam 0.15mg/kg) if there is a bizarre behaviours and consult.

Medication and treatment options documented in the workbook include the following regimen examples:

Treatment: no. Formulary examples/brands in source: fleming, paralief for children, panadol advance.

Treatment: amoxycillin+ clavulanate over 12years 375-750mg, below 12 years 25-50mg/kg/day, paracetamol 10mg/kg/dose. Formulary examples/brands in source: fleming, paralief for children, panadol advance. Treatment: paracetamol 10mg/kg/dose, ceftriaxone 50mg/kg/dose, transfuse if symptomatic, ranferon. Formulary examples/brands in source: Rocephin,paraconica 1g. Treatment: amoxycillin+ clavulanate over 12years 375-750mg, below 12 years 25-50mg/kg/day, paracetamol 10mg/kg/dose, artemether/lumefantrine per kg body weight. Formulary examples/brands in source: fleming, paralief for children, panadol advance, ACTM 20/120. Treatment: Artesunate 2.4mg/kg loading dose +paracetamol 10 mg/kg/dose, ceftriaxone 50mg/kg/dose. Formulary examples/brands in source: Rocephin,paraconica 1g, Artesun. Treatment: Artesunate2.4mg/kg/dose, paracetamol, 10mg/kg/dose, ceftriaxone 100mg/kg/dose transfuse if < 5g/dl and give ranferon at HB> 6g/dl. Formulary examples/brands in source: Rocephin,paraconica 1g, Artesun. Treatment: Iv Ceftriaxone 100mg/kg BD Max dose 2g, paracetamol 10mg/kg/dose. Formulary examples/brands in source: Rocephin,paraconica 1g. Treatment: paracetamol10mg/kg/dose, paracetamol,10mg/kg/dose.

Condition 3: Elevated BPs in Pregnancy

Source rows integrated: 134

Presentation: ELEVATED BP $\geq$ 140/90mmHg. Clinical features: WITH ANY OF:signs of target organ damage (Severe headache with blurring of vision, Epigastric pain, Oliguria , signs , liver tenderness. Initial investigations: FHG + Urinalysis +U/E/C+LFTs + Coagulation profile. Key result pattern: Normal FHG + Normal Transaminases + Normal U/E/C + Normal coagulation profile + No Proteinuria. Additional tests: Obstetric U/S + Uric acid + PBF. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: Severe Pre - Eclampsia; Gestational hypertension/Mild Pre Eclampsia; Gestational hypertension + Anaemia/ Mild Pre Eclampsia; Severe Pre eclampsia; Severe Pre eclampsia , Anaemia; severe Pre Eclampsia + Anaemia; Gestational hypertension/ Mild Pre Eclampsia; severe Pre Eclampsia, +Anaemia.

Initial management pathway: reassure patient, place the patient in left lateral position, restrict fluids to 80mls per hr of ringers lactate avoid DNS or Dextrose.; reassure patient, place the patient in left lateral position.; reassure patient, place the patient in left lateral position.,. Operational plan points include: Admit and call gynaecologist immediately.

Medication and treatment options documented in the workbook include the following regimen examples:

Treatment: if BP > 160/110 mm/hg give IV hydralazine 5mg slowly over 10 minutes , Give loading 4 GM of 20% MGSO₄ over 15 minutes then continue with 1 GM hourly for 24 hours as you monitor the respiratory rate, urine out put and patellar reflexes. And continue for 24 hours after delivery whichever comes first.. Formulary examples/brands in source: Apresolin 20mg inj, Nalepsin 4gm injection. Formulary examples/brands in source: Apresolin 20mg inj, Nalepsin 4gm injection.

Treatment: Methyldopa 250 mg TID. Formulary examples/brands in source: Aldomet 250mg tablets.
Treatment: Methyldopa 250 mg TID. Formulary examples/brands in source: Aldomet 205mg tablets.
Treatment: Methyldopa 250mg TID. Formulary examples/brands in source: Aldomet 250mg tablets.
Treatment: Methyldopa 250mgTID. Formulary examples/brands in source: Aldomet 250mg tablets.
Treatment: No treatment.

Condition 4: HEAD INJURY

Source rows integrated: 6

Presentation: Trauma to the head. Clinical features: WITH:-Convulsions, Confusion, Ottorrhoea/Rhinorrhoea, Penetrating, palpable #/Deformity, Blurring of vision, vomiting Anisocoria, Lateralizing signs , extremities of age < 5 years > 60 years. Initial investigations: Order an urgent CT scan Head,. Additional tests: U/E/C, RBS, LFTS FHG,Alcohol toxicity levels.. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: Dx- Mild Head injury; Dx- Moderate Head injury; Dx- severe Head injury; Dx: Mild Head Injury; Dx: Moderate Head Injury; Dx: Severe Head Injury.

Initial management pathway: Assess Airway , Circulation, Breathing, Disability and Exposure(ACBDE) Arrest Bleeding(If Present) , if in shock see appendix (x). Operational plan points include: Admit and immediate surgical consult; Admit for Observation.

Medication and treatment options documented in the workbook include the following regimen examples: Treatment: IV ceftriaxone 100mg/kg/dose , Paracetamol 10mg/kg/dose, Phenytoin 20mg/kg to run over 15 mins with ECG & BP monitoring, TT 0.5mg IM SAT in > 5 years old. Formulary examples/brands in source: Rocephin 2gm,Paraconica 1gm, Epanutin 250mg inj, Tetenus injection.

Condition 5: ABDOMINAL INJURY

Source rows integrated: 4

Presentation: Abdominal Injury. Clinical features: WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg. Initial investigations: Focused Assessment Sonography in Trauma (FAST U/S) as you consult. Additional tests: FHG + U/E/C + GXM + LFTs. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: Blunt Abdominal injury with SHOCK; Penetrating Abdominal injury with SHOCK; Blunt Abdominal injury; Penetrating Abdominal injury.

Initial management pathway: See appendix (x) for hypovolemic shock resuscitation; See appendix (x) for hypovolemic shock resuscitation, DO NOT REMOVE any impinged penetrating object, if no impinged object PACK WITH STERILE GAUZE; Reassure patient, maintenance IVF,; Reassure patient, maintenance IVF,DO NOT REMOVE any impinged penetrating object, if no impinged object PACK WITH STERILE GAUZE. Operational plan points include: Admit and immediate surgical consult; Admit for observation.

Medication and treatment options documented in the workbook include the following regimen examples: Treatment: IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose.

Formulary examples/brands in source: Rocephin 1gm injection, Flagyl infusion, paraconica 1gm. Treatment: IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose, TT 0.5mg IM SAT in > 5 years old. Formulary examples/brands in source: Rocephin 1gm injection, Flagyl infusion, paraconica 1gm tetanus injection.

Condition 6: CHEST TRAUMA

Source rows integrated: 6

Presentation: Trauma to the Chest + Difficulty in breathing.. Clinical features: WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg. Initial investigations: chest x-ray. Additional tests: FHG, U/E/C. Abdominal scan especially if # is in 11th and 12th ribs or with associated abdominal tenderness. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: Dx- Blunt chest trauma in shock.; Dx- Blunt chest trauma; Dx- Blunt chest trauma with pneumothorax; Dx- Blunt chest trauma with hemothorax.

Initial management pathway: See appendix (x) for hypovolemic shock resuscitation, give oxygen, Do Assess and manage Airway, Circulation, Breathing, Disability. As you consult .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK; Reassure patient, maintenance IVF, .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK; See appendix (x) for hypovolemic shock resuscitation, give oxygen, Do Assess and manage Airway, Circulation, Breathing, Disability, insert a large bore cannula at 2nd intercostal space mid clavicular line, .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK; Reassure patient, maintenance IVF, give oxygen, Do Assess and manage Airway, Circulation, Breathing, Disability, insert a large bore cannula at 2nd intercostal space mid clavicular line urgent surgical review, .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK; See appendix (x) for hypovolemic shock resuscitation, give oxygen, Do Assess and manage Airway, Circulation, Breathing, Disability as you consult for chest tube insertion, .IF PENETRATING OBJECT INSITU, DO NOT REMOVE. ARREST ANY BLEEDING BY PUTTING A STERILE PACK. Operational plan points include: Admit and consult immediately.

Medication and treatment options documented in the workbook include the following regimen examples: Treatment: IV ceftriaxone 50mg/kg/dose, Iv Metronidazole 7.5mg/kg/dose, Paracetamol 10mg/kg/dose. Formulary examples/brands in source: Rocephin 1gm injection, Flagyl infusion, Paraconica 1gm injection.

Condition 7: EPIGASTRIC PAIN

Source rows integrated: 81

Presentation: EPIGASTRIC PAIN. Clinical features: bloody Vomitus WITH signs of SHOCK (Confusion, reduced consciousness, palpitations, sweating, Severe pallor , weak thready pulse, fast breathing, HR > 100bpm, oliguria< 0.5mls/kg/hr RR- 22, BP- (90mmHg) anuria, BP - SBP <90mmHg. Initial investigations: FHG + U/E/C + LFTs + H- Pylori Ag. Key result pattern: Normal FHG + Normal Transaminases + Normal U/E/C + H-Pylori -VE. Additional tests: Endoscopy, GXM. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: probable dx- Upper GI bleeding. With shock; probable dx- Upper GI bleeding, Anaemia With shock; Probable dx- Upper GI bleeding with shock, R/O a coagulopathy; probable dx- Liver failure, oesophageal varices.; probable dx- Liver failure, oesophageal varices. Coagulopathy.; probable dx- Shock, R/O AKI; probable dx- Shock, R/O AKI, Anaemia.; probable dx-shock,R/O AKI,R/O coagulopathy.

Initial management pathway: See appendix (x) for hypovolemic shock resuscitation; start IV normal saline as you consult; start on pain relieve. Operational plan points include: Admit and urgent surgical consult; assess, see as an OPD and a TCA of 7 days.

Medication and treatment options documented in the workbook include the following regimen examples:

Treatment: start on fluid resuscitation (see appendix x), iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Tranexamic acid 10mg/kg/dose/TID. Formulary examples/brands in source: Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj, Hemsamic 500mg injection. Treatment: Start on, iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Tranexamic acid 10mg/kg/dose TID. Formulary examples/brands in source: Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj, Hemsamic 500mg injection. Treatment: start on fluid resuscitation (see appendix x), iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,. Formulary examples/brands in source: Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj. Treatment: Start on, iv clarithromycin ,1 month - 12 years 7.5mg/kg BID DO NOT EXCEED 500MG > 12 years 500mg BID IV Augmentin < 12 years 25mg/kg/day TID >12 years 45mg/kg TID IV Ranitidine 50mg QID > 16 Years > 16 years 2mg /kg /day TID ,Tranexamic acid 10mg/kg/dose TID. Formulary examples/brands in source: Klacid 500mg injection,Augmentin 1.2gm injection, Zantac 50mg inj, hemsamic 500mg inj. Treatment: > 12 years esomeprazole 20mg BID /day , paed 3.5kg : 2.5mg P.O /day , 3.5kg-7.5kg: 5mg PO /day, > 7.5 kg 10mg PO /day. Formulary examples/brands in source: Nexium 20mg tablets. Treatment: Start on Eso kit and paracetamol 10mg/kg/dose. Formulary examples/brands in source: Esclam kit, Panadol Advance. Treatment: Start n Eso kit and paracetamol 10mg/kg/dose. Formulary examples/brands in source: Esclam kit, Panadol Advance. Treatment: Start on Eso kit and paraceamol 10mg/kg/dose. Formulary examples/brands in source: Esclam kit, Panadol Advance.

Condition 8: WHEEZE

Source rows integrated: 4

Presentation: Wheezing with Difficulty in Breathing. Clinical features: WITH ANY OF: While walking, Talks in Alertness, Can lie down, SentencesMay be agitated, Increased Respiratory rate,no use of accessory muscles,wheeze often only end expiratory, Pulse Rate <100, Pulsus paradoxus, Absent/ <10 mm Hg. Initial investigations: FHG. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: MILD ASTHMATIC ATTACK; MODERATE ASTHMATIC ATTACK; SEVERE ASTHMATIC ATTACK; ACTUAL/ IMPENDING RESPIRATORY FAILURE.

Initial management pathway: Reassure patient/ guardian, administer oxyegen to maintain SPO2 > 90%, Give health education See appendix (XII); INTUBATE PATIENT Reassure patient/ guardian, administer oxyegen to maintain SPO2 > 90%, Give health education See appendix (XII). Operational plan points include: TREAT AS OP & TCA 24HRS,; ADMIT & CALL PHYSICIAN IMMEDIATELY; ADMIT IN CRITICAL CARE & CALL

PHYSICIAN IMMEDIATELY.

Medication and treatment options documented in the workbook include the following regimen examples:
Treatment: Nebulize with Salbutamol 1Mo - 11 yrs 2.5mg, > 11 years 5mg can be repeated up to 3 doses in the 1st hour + Prednisolone 1mg/kg daily for 1/52. Formulary examples/brands in source: Ventolin nebulizing solution, prednisolone 5mg tablets. Treatment: Nebulize with Salbutamol 1Mo - 11 yrs 2.5mg, > 11 years 5mg can be repeated up to 3 doses in the 1st hour + IV Ipratropium 200mg + IV Hydrocortisone 50mg(1-5 yrs), 100mg (6-12 yrs),. Formulary examples/brands in source: Ventolin nebulizing solution, Atrovent 250mg nebulules, Hydrocortison 100mg inj.

Condition 9: BURNS

Source rows integrated: 15

Presentation: PATIENT WITH BURNS. Clinical features: Partial thickness burns greater than 10% total body surface area (TBSA). 2. Burns that involve the face, hands, feet, genitalia, perineum, or major joints. 3. Third degree burns in any age group. 4. Electrical burns, including lightning injury. 5. Chemical burns. 6. Inhalation injury. 7. Burn injury in patients with preexisting medical disorders that could complicate management, prolong recovery, or affect mortality. 8. Any patient with burns and concomitant trauma (such as fractures) in which the burn injury poses the greatest risk of morbidity or mortality. . 9. Extremes of age <5 & >60 10. Burn injury in patients who will require special social, emotional, or rehabilitative intervention 12. Circumferential burns. Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential and pathway framing from the source includes: MILD BURN INJURY; MODERATE BURN INJURY; SEVERE BURN INJURY.

Initial management pathway: IN SEVERE BURNS DO ACBE remove all burned clothing, immerse the site in cold water for 30 minutes,douse with cool water, apply clean wraps on burned area,administer tetanus prophylaxis, gently cleanse the burn with 0.25% chlorhexidine solution, DO NOT use alcohol-based solutions,Gentle scrubbing will remove the loose necrotic tissue. Apply a thin layer of antibiotic cream (silver sulfadiazine). Dress the burn with petroleum gauze and dry gauze; IN SEVERE BURNS DO ACBE remove all burned clothing, immerse the site in cold water for 30 minutes,douse with cool water, apply clean wraps on burned area,administer tetanus prophylaxis, gently cleanse the burn with 0.25% chlorhexidine solution, DO NOT use alcohol-based solutions,Gentle scrubbing will remove the loose necrotic tissue. Apply a thin layer of antibiotic cream (silver sulfadiazine). Dress the burn with petroleum gauze and dry gauze SEE APPENDIX 12 FOR FLUID MANEGEMENT IN BURNS. Operational plan points include: Treat as OP & TCA 3 days; Admit for Surgical review.

Medication and treatment options documented in the workbook include the following regimen examples:
Treatment: Fluloxacillin 50mg/kg/dose + Paracetamol 10mg/kg/dose + silver sulphur diazine cream. Formulary examples/brands in source: Adult Floxapen 500mg caps, Panadol advance, silverex 500gm, 25gm, Paed floxapen 125mg syrup, panadol baby and infant, silverex 500gm, 25gm. Treatment: IV Fluloxacillin 50mg/kg/dose + IV Metronidazole 7.5mg/kg + Pethidine 50mg IM + Tetracycline eye ointment in facial burns, Pethidine 50mg IM,Omeprazole 4mg/kg/dose,silver sulphur diazine cream. Formulary examples/brands in source: Floxapen 500mg inj, Flagyl infusion, pethidine 50mg, tetracycline eye ointment,losec 40mg injection,silverex cream 500gm. Treatment: IV Fluloxacillin 50mg/kg/dose + IV Metronidazole 7.5mg/kg + Pethidine 50mg IM + Tetracycline eye ointment in facial burns, Pethidine 50mg IM,Omeprazole 4mg/kg/dose, silver sulphur diazine cream. Formulary examples/brands in source: Floxapen 500mg inj, Flagyl infusion, pethidine 50mg, tetracycline eye ointment,losec 40mg injection,silverex cream 500gm.

Condition 10: HYPONATREMIA

Source rows integrated: 24

Clinical pathway selection should begin with severity assessment, immediate threat recognition, and early stabilization when shock or respiratory compromise is suspected.

Differential diagnosis should be prioritized using presenting features, investigation pattern, and response to initial supportive care.

Initial management pathway: basic supportive measures and close monitoring. Operational plan points include: admit for further evaluation where instability or high-risk findings are present.

Medication and treatment options documented in the workbook include the following regimen examples: Treatment: (Serum Na < 130 mmol/L). Treatment: signs and symptoms. Treatment: nausea, malaise, lethargy, decreased level of consciousness, headache, if severe (seizures and coma), overt neurologic symptoms (<115 mEq/L). Treatment: CLASSIFICATION. Treatment: 1. Hypovolemic-decrease in total body water with greater decrease in total body sodium. Treatment: 2. Euvolemic- normal body sodium with increase in total body water. Treatment: 3. Hypervolemic- increase in total body sodium with increase in total body water. Treatment: LAB WORKUPS.

Appendix Integration Notes

Appendix (i):

TREATMENT OF HYPERKALAEMIA Medication | Dose | Action and Duration stop all potassium intake Step 2 Cation-exchange resin sodium or calcium polystyrene sulphonate | 0.5-1.0g/kg orally or high enema | removes K⁺ from body. Lasts 2 hours step 3 10 % calcium gluconate | 0.5-1.0 mg/kg IV over 5-10 minutes | Neutralises K carditoxic effects. lasts 30-60 minutes. step 4

Appendix (ii):

DKA MANAGEMENT CLINICAL HISTORY POLYURIA, POLYDIPSIA, WEIGHT LOSS(weigh) ABDOMINAL PAINS TIREDNESS VOMITING Assess DEHYDRATION (> 5 %) Deep sighing respiration (Acidotic breathing kussmaul's breathing) smell of ketones (Sweet fruity breath) ketones in urine (> ++) elevated blood sugar (> 11 mmol/L) Acidaemia (PH < 7.3) Take BGA (bicarbonate < 15 | assessing dehydration % Moderate 4-6 % Delayed capillary refill > 2 sec, Increased respiratory rate . Severe dehydration >= 3 sec mottled skin signs of shock Deep acidotic breathing, decreased tissue Turgor ASSESS FOR SHOCK | fluid requirement = amount of deficit + amount of maintenance. Deficit = estimated % dehydration x bodyweight. Approximate maintenance per weight 3-9 kgs = 80 mls /Kg/Day, 10 -19 Kg 70 mls/Kg/Day , 20 - 29 Kg = 60 mls /Kg/ day, 30- 50 Kgs 50 mls/Kg/ Day Over 50 Kgs = 35 mls/ Kg /Day (reduced peripheral pulses, reduced consciousness level, coma) 1. Airway/NG tube 2. Breathing (100% oxygen) 3. Circulation Saline 0.9 % 10mls/kg over 10- 30 minutes until circulation is restored. May be repeated calculate fluid requirement to be corrected over the next 48 hours use saline 0.9% Give KCl 20 -30 mmol/hr after the the first 30- 60 minutes (KCl Preparation 1 ml of 15 % contains 2 mmol K, 1 g KCL = 13 mmol K) if Potassium levels are normal or below normal, or if insulin and sodium bicarbonate have been given before admission. If levels are high, wait until they normalize or go down.

appendix iv:

FLUID MANAGEMENT IN NEONATES DAY 1: 60mls/kg/day DAY 2: 90mls/kg/day DAY 3: 120mls/kg/day. Increase 30mls/kg/day. And encourage breastfeeding. FLUID MANAGEMENT IN CHILDREN NO DEHYDRATION, USE PLAN A SIGNS AND SYMPTOMS OF SOME DEHYDRATION: Either 1 of below signs and symptoms. · Restlessness/irritability. · Lethargy.

APPENDIX (vii):

HYPERNATRAEMIA TREATMENT (Serum Na > 150 mmol/L) common causes 1.hypernatraemic dehydration e.g Gastroenteritis 2.Salt poisoning TREATMENT Hypotonic solutions HSD at the rate of 0.5 -1.0 mmol/hour over 24 hours Do 4 hourly check until they fall to below 150 mmol/L hourly weighing

Sheet2:

RISK FACTORS FOR PPH PRENATAL | ANTEPARTUM | DURING ADMISSION | INTRAPARTUM MEDIUM | HIGH RISK | MEDIUM | HIGH RISK Suspected previa/increta/percreta | Placent previa | prior C/S (MEDIUM) | placenta previa | Chorioamnionitis | NEW Active bleeding BMI> 50 | placenta accreta | Uterine surgery | suspected Accreta/Percreta | prolonged oxytocin > 24 hours | 2/> medium risk factors(admission OR intrapartum) prior classical C/S | multiple Gestation | Platelet < 70,000 | prolonged 2nd stage prior myomectomy | > 4 prior births | Active bleeding large myomas | known coagulopathy EFW> 4000g | 2/> medium risk Obesity BMI> 40

End of Handbook